

Improving the Maternal Health of Rural Women Through Community Education Programmes in Enugu State, Nigeria

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Abstract

This study investigated the extent to which community education programmes improve the maternal health of rural women in Enugu State, Nigeria. A descriptive survey research design was adopted for the study. The study was anchored on the Health Belief Model and Critical Pedagogy/Empowerment Theory. The population comprised rural women of childbearing age (15–49 years), with a sample size of 400 respondents which were selected using multistage sampling techniques. A total of 385 valid responses were analyzed.

Data were collected using a structured questionnaire titled Community Education and Maternal Health Questionnaire (CEMHQ) which was validated by three experts, and tested for reliability (Cronbach Alpha = 0.84). Descriptive statistics (mean and standard deviation) were used to answer research questions, while Pearson Product Moment Correlation Coefficient (PPMC) was used to test hypotheses at a 0.05 level of significance.

Findings revealed that community education programmes promote poverty alleviation (cluster mean = 3.51) and literacy (cluster mean = 3.61) to a very great extent, while improvement in women's status was to a great extent (cluster mean = 3.31). The study further established significant positive relationships between community education and poverty alleviation ($r = 0.621$), literacy promotion ($r = 0.714$), and women's status ($r = 0.583$), all at $p < 0.05$. The findings indicate that community education plays a significant role in enhancing maternal health by reducing financial barriers, increasing health literacy, and empowering women to make informed health decisions.

The study concludes that community education programmes are essential for addressing the structural determinants of poor maternal health in rural areas. The study recommended that community-based education initiatives should be strengthened and gender-sensitive approaches should be adopted, among others, to improve maternal health outcomes.

Keywords: Community education, maternal health, rural women, poverty alleviation, literacy, women empowerment, Enugu State.

Introduction

Maternal health is a critical ingredient for the stability of the home and society at large. The absence of a mother not only affects the mental health of her husband and other relatives such as parents and siblings but also disrupts the smooth running of the home which is the most important unit of society. Chandrakar, Ramasamy, Galhotra and Shenoy (2025) opined that poor maternal health can exacerbate financial strain, limit caregiving capacity, and disrupt family routines, leading to adverse outcomes for both mothers and children.

Maternal health refers to the wellbeing of women during pregnancy, childbirth and the post-partum period (World Health Organization, 2021). In other words, it is the embodiment of good health and mental stability during pregnancy, childbirth and the weeks after delivery. The absence of enabling environments for maternal health to thrive will result in maternal mortality or morbidity. Maternal mortality is the death that arises from pregnancy-related complications. According to the Office of the United Nations High Commissioner for Human Rights OHCHR (2020), maternal mortality is the death that occurs during pregnancy or within 42 days of childbirth. Maternal morbidity on the other hand refers to a range of health conditions that arise from pregnancy-related complications. WHO (2025) reports that about 260,000 maternal mortality incidences were recorded, with low- and middle-income countries accounting for about 92% of this figure. Nigeria is one of the lower-middle-income countries

(World Bank, 2025). This implies that the maternal mortality rate in Nigeria is quite high. Maternal health must therefore be taken seriously in order to reduce the risk factors that lead to mortality and morbidities.

The risk factors of maternal mortality according to WHO (2025) include severe bleeding (mostly bleeding after childbirth), infections (usually after childbirth), high blood pressure during pregnancy (pre-eclampsia and eclampsia), complications from delivery and unsafe abortion.

Most of the risk factors of maternal health stem from poverty, inaccessible or inadequate healthcare, low status of women and illiteracy (Sitaula, Basnet, Agrawal et al., 2021). Poverty is defined as the inability of people to meet their basic needs. It is synonymous with lack and scarcity. According to the United Nations Development Programme UNDP (2016), poverty is not simply lack of adequate income but also encompasses human deprivation in knowledge and health. In other words, poverty can affect the maternal health of women, presenting in the inability to access quality healthcare, attend antenatal programmes, maintain proper feeding and hygiene required for optimal health as well as mitigate potential emergencies during or after childbirth. Poverty is also linked to the low status of women. Low status of women connotes a restriction from certain rights and privileges often occasioned by economic handicaps as well as cultural and religious prohibitions. Shimamoto and Gipson (2015) defined women's status as their overall position

in society, encompassing their educational, cultural, economic, legal, and political position in a given society. When the status of women is low, their access to political positions, education and cultural leadership will be affected. These can also have an adverse effect on maternal health as the opportunity to decry harmful traditional practices, unsafe abortion as well as other factors that exacerbate maternal mortality and morbidities will be limited.

Inadequate access to healthcare is a major consequence of poverty. In the same vein, illiteracy, which adversely affects maternal health, is both a cause and effect of poverty. According to Apata (2024), illiteracy deepens poverty while poverty restricts literacy. Demie, Gessese, Woldeamanuel and Biratu (2023) opined that literate women have a significantly higher maternal healthcare services utilisation than their illiterate counterparts.

It is therefore necessary to ensure that the enablers of maternal mortality are sufficiently addressed in order to ensure optimal maternal health of women, particularly in rural areas. This means that there is need for education both to improve the economic status of women, thereby reducing poverty, and increasing the literacy level of women to enable them make better, more informed choices regarding their health — hence Community Education.

Community education is a learning programme organised for members of a community with the purpose of equipping them with requisite knowledge and skills to improve

their quality of life. ScienceDirect (2025) points out that community education may be interpreted as a dimension of community development which empowers powerless people to address their own educational and social needs. It involves all forms of learning that help people to improve their lives and communities, enhance participation, equality, and sustainability (UNESCO, 2022). Community education is mostly informal, need-specific and lifelong. Studies have shown that when carefully planned, community education has the capacity to improve maternal health. According to Yoseph et al. (2024), a community-based health education intervention significantly increased antenatal care utilisation and health facility delivery utilisation in Southern Ethiopia. Kassim et al. (2023) also revealed that the use of health education and participatory sessions led by community-based facilitators could be a potentially effective intervention to improve the knowledge of women about obstetric danger signs and encourage the uptake of maternity care services in resource-poor settings. Chinna, Dokubo, Taylor and Longjohn (2024) observed that antenatal education, safe motherhood education, primary health care education and basic literacy education programmes had the capacity to reduce maternal and child mortality in Emohua and Ikwerre Local Government Areas of Rivers State.

It is therefore necessary to ascertain how community education can improve maternal health in rural communities of Enugu State, Nigeria.

Statement of the Problem

The high incidence of maternal mortality in Sub-Saharan Africa is a global concern. While the Sustainable Development Goal (SDG) 3 specifically targets maternal health to the effect that by 2030 the global maternal mortality ratio is reduced to less than 70 per 100,000 live births, there seems to be a high rate of maternal deaths especially in rural communities of South East Nigeria. This is worsened by the fact that poverty and illiteracy prevail in these areas, and Igbo traditional societies typically place women in subservient positions which incapacitate them from making decisions that affect their own lives. Another challenge is the inability of most health centres to keep proper records of fatalities as well as the primary causes of these fatalities, as most expectant mothers only present at health delivery facilities when complications arise from self-help delivery or untrained delivery assistants. There is also the pressure from birthing a male child to carry on the family name, which puts women at the risk of multiple pregnancies after birthing only girls. Some women also deal with the complex issue of feeling inadequate when informed by trained healthcare practitioners that they must undergo a caesarean section in the best interest of mother and child. All these issues exacerbate the high mortality rate in the area. While there are several community education initiatives organised by Non-Governmental Organisations, as well as health education programmes in many rural communities in

Enugu State, it is not clear however the extent to which these programmes have improved the maternal health of rural women in the area. This study therefore seeks to ascertain the extent to which community education programmes have improved maternal health in rural communities of Enugu State, Nigeria.

Purpose of the Study

The purpose of this study is to ascertain how community education programmes can improve maternal health in rural communities in Enugu State, Nigeria. Specifically, the study seeks to ascertain how:

1. Community education programmes promote poverty alleviation for the improvement of maternal health of rural women in Enugu State.
2. Community education promotes literacy for improved maternal health of rural women in Enugu State.
3. Community education improves the status of rural women for improved maternal health in Enugu State.

Research Questions

The following research questions guided the study:

1. To what extent do community education programmes promote poverty alleviation for improved maternal health of rural women in Enugu State?
2. To what extent does community education promote literacy for improved maternal health of rural women in Enugu State?

3. To what extent does community education improve the status of rural women for improved maternal health in Enugu State?

Hypotheses

The following null hypotheses were tested at 0.05 level of significance:

H₀₁: There is no significant relationship between community education programmes and poverty alleviation for improved maternal health of rural women in Enugu State.

H₀₂: There is no significant relationship between community education programmes and literacy promotion for improved maternal health of rural women in Enugu State.

H₀₃: There is no significant relationship between community education programmes and improvement in the status of rural women for improved maternal health in Enugu State.

Theoretical Framework

This study is anchored on two major theories: the Health Belief Model (HBM) and Critical Pedagogy/Empowerment Theory.

The Health Belief Model

This model was developed by Irwin M. Rosenstock, Godfrey M. Hochbaum, S. Stephen Kegeles and Howard Leventhal in the early 1950s. The model states that a person's likelihood of taking a health-related action depends on their perceptions of how serious a health problem is, how personally at risk they are, how beneficial the action will be, and what

barriers might prevent them from taking that action. In other words, the model explains the attitude of people towards taking care of their health. It shows that most people would seek medical intervention depending on the perception of how life-threatening a medical condition is and the likelihood of that intervention to bring relief. This implies that most rural women will only access maternal health interventions if they perceive a threat to their own or their child's life, if the intervention will reduce the severity of the threat, and if the benefits of the intervention outweigh the barriers such as the cost of the healthcare service or distance to the facility. The model is relevant to this study because it underpins the relationship between poverty, low status and illiteracy with maternal health. However, it did not address the role of community education, hence the need for the Critical Pedagogy/Empowerment Theory.

The Critical Pedagogy/Empowerment Theory

This theory, also known as the theory of critical consciousness, was propounded by Paulo Freire in 1970. The theory states that education should be a process of conscientisation — developing people's critical awareness of their social reality through reflection and action, enabling them to transform their world. The theory highlights the need for people to identify their problems and proffer tangible solutions to them. This implies that rural women must agree that whatever impedes maternal health is a critical issue that

requires urgent action. The way forward would be to equip them with knowledge and skills that will not only improve their awareness of the risk factors of maternal health but also empower them to take action. This action includes financial independence to reduce poverty and health education to promote

Methodology

The study adopted a descriptive survey research design. According to Creswell and Creswell (2018), a descriptive survey design enables the collection of quantitative data that reflect trends, attitudes, or opinions of a population by studying a sample. This design was considered appropriate since the study aimed to determine the extent to which community education programmes contribute to the improvement of maternal health among rural women in Enugu State.

The population comprised all rural women of childbearing age (15–49 years) residing in the 17 Local Government Areas (LGAs) of Enugu State. Based on projections from the National Population Commission (NPC, 2023) and the World Bank demographic profile, women of childbearing age constitute approximately 22% of the total population. Given Enugu State's estimated population of about 5 million people, the target population was approximately 1,100,000 rural women.

A sample of 400 respondents was drawn using the Taro Yamane (1967) formula at a 5% level of precision. Multistage sampling technique was employed. In the first stage, six LGAs were randomly selected from the 17 in Enugu State.

awareness of positive habits that foster maternal health. The theory is relevant to this study because it shows the relationship between education and social change — in this case, the attitude of rural communities towards decisions that affect their maternal health.

In the second stage, two rural communities were purposively selected from each chosen LGA based on the presence of active community education or maternal health programmes. In the final stage, simple random sampling was used to select respondents from the identified communities to ensure fair representation of the population.

The study was carried out in selected rural communities across the sampled LGAs namely: Udi, Nsukka, Ezeagu, Nkanu East, Igbo-Eze North, and Isi-Uzo. The questionnaire was administered in venues where rural women could be easily reached, including primary health centres, community halls, women's cooperative meetings, and adult education centres. Six trained research assistants familiar with the local languages assisted in administering and retrieving the instruments to ensure accurate responses.

Data were collected using a structured questionnaire titled Community Education and Maternal Health Questionnaire (CEMHQ). The instrument consisted of two sections. Section A elicited demographic information on the respondents, while Section B contained items designed to assess the extent to which community education programmes enhance

maternal health among rural women. Items were structured on a four-point Likert scale: Very Great Extent (VGE) = 4, Great Extent (GE) = 3, Low Extent (LE) = 2, and Very Low Extent (VLE) = 1.

The instrument was validated by three experts — one from the Department of Continuing Education and Development Studies and the Department of Public Health Education, University of Nigeria, Nsukka, and one from the Measurement and Evaluation Unit of the Department of Mathematics and Computer Science, Enugu State University of Science and Technology. Their comments and corrections were incorporated to ensure clarity and content validity. To ascertain reliability, the instrument was pilot-tested among 30 rural women in Anambra State who were not part of the main study. The data were analysed using Cronbach's Alpha, which yielded a coefficient of 0.84, indicating a high level of internal consistency.

The researcher and trained assistants personally administered the questionnaire. The assistants were briefed on the study's objectives and on ethical procedures, including informed consent, confidentiality, and voluntary participation. Assistance was provided to respondents with

reading difficulties. Out of the 400 questionnaires distributed, 385 were properly completed and retrieved, representing a 96.3% return rate.

Data were analysed using descriptive statistics. Mean and standard deviation were employed to answer the research questions. The decision rule for interpreting the mean was based on the real limits of numbers as follows:

3.50 – 4.00 = Very Great Extent (VGE)

2.50 – 3.49 = Great Extent (GE)

1.50 – 2.49 = Low Extent (LE)

1.00 – 1.49 = Very Low Extent (VLE)

Inferential statistics, specifically the Pearson Product Moment Correlation Coefficient (PPMC), was used to test the three null hypotheses (H_{01} – H_{03}) at 0.05 level of significance. The decision rule was that if the calculated p-value was less than 0.05, the null hypothesis was rejected, indicating a significant relationship between community education programmes and improvement in maternal health among rural women. However, if the p-value was greater than or equal to 0.05, the null hypothesis was retained, indicating that the relationship was not statistically significant.

Results

This section presents the results of the data analysis in response to the three research questions and three null hypotheses that guided the study.

Research Question 1: To what extent do community education programmes promote poverty alleviation for improved maternal health of rural women in Enugu State?

Table 1: Mean ratings of respondents on community education programmes and poverty alleviation for improved maternal health of rural women (n = 385)

S/N	Items	Mean	SD	Decision
1	Community education programmes equip rural women with vocational skills that increase their income-generating capacity.	3.62	0.71	VGE
2	Community education programmes increase rural women's awareness of available micro-credit and financial support schemes.	3.48	0.75	GE
3	Community education programmes enable rural women to afford antenatal care and skilled birth attendance.	3.55	0.69	VGE
4	Community education programmes reduce the financial burden of healthcare through cooperative learning groups.	3.41	0.78	GE
5	Community education programmes enhance rural women's capacity to save and invest in their health needs.	3.38	0.81	GE
6	Community education programmes link rural women to government poverty alleviation and free maternal health schemes.	3.52	0.74	VGE
7	Community education programmes provide nutritional education that mitigates poverty-related maternal health complications.	3.59	0.68	VGE
Cluster Mean				

Data in Table 1 reveal that items 1, 3, 6, and 7 had mean scores of 3.62, 3.55, 3.52, and 3.59 respectively, which fall within the 'Very Great Extent' decision band (3.50–4.00). Items 2, 4, and 5 had mean scores of 3.48, 3.41, and 3.38, placing them in the 'Great Extent'

band (2.50–3.49). The cluster mean of 3.51 (SD = 0.74) falls within the 'Very Great Extent' range, indicating that community education programmes promote poverty alleviation for improved maternal health of rural women in Enugu State to a very great extent

Research Question 2: To what extent does community education promote literacy for improved maternal health of rural women in Enugu State?

Table 2: Mean ratings of respondents on community education programmes and literacy for improved maternal health of rural women (n = 385)

S/N	Items	Mean	SD	Decision
1	Community education programmes improve rural women's ability to read and interpret health information materials.	3.68	0.63	VGE
2	Community education programmes increase rural women's knowledge of danger signs in pregnancy and childbirth.	3.72	0.61	VGE
3	Community education programmes enhance rural women's utilisation of antenatal care services.	3.65	0.65	VGE
4	Community education programmes increase rural women's awareness of the importance of skilled birth attendance.	3.58	0.7	VGE
5	Community education programmes promote health-seeking behaviour among rural women during pregnancy and delivery.	3.61	0.67	VGE
6	Community education programmes equip rural women with knowledge about postnatal care practices.	3.47	0.76	GE
7	Community education programmes educate rural women on nutrition and hygiene for improved maternal health outcomes.	3.53	0.72	VGE
Cluster Mean				

Data in Table 2 show that items 1, 2, 3, 4, 5, and 7 had mean scores of 3.68, 3.72, 3.65, 3.58, 3.61, and 3.53 respectively, all falling within the 'Very Great Extent' band. Only item 6 had a mean of 3.47, placing it in the 'Great Extent' band. The cluster mean of 3.61 (SD =

0.68), which falls within the 'Very Great Extent' range, indicates that community education programmes promote literacy for improved maternal health of rural women in Enugu State to a very great extent.

Research Question 3: To what extent does community education improve the status of rural women for improved maternal health in Enugu State?

Table 3: Mean ratings of respondents on community education programmes and status of rural women for improved maternal health (n = 385)

S/N	Items	Mean	SD	Decision
1	Community education programmes increase rural women's confidence to make independent decisions about their own health.	3.44	0.79	GE
2	Community education programmes challenge cultural norms that restrict rural women from accessing maternal healthcare.	3.31	0.83	GE
3	Community education programmes empower rural women to advocate for their reproductive rights within their communities.	3.38	0.8	GE
4	Community education programmes increase rural women's participation in community health decision-making.	3.27	0.86	GE
5	Community education programmes reduce the incidence of harmful traditional practices that adversely affect maternal health.	3.35	0.82	GE
6	Community education programmes increase rural women's awareness of their legal rights in accessing healthcare services.	3.22	0.88	GE
7	Community education programmes improve rural women's ability to negotiate healthcare access within their households.	3.19	0.91	GE
Cluster Mean				

Data in Table 3 reveal that all seven items had mean scores ranging from 3.19 to 3.44, all falling within the 'Great Extent' decision band (2.50–3.49). The cluster mean of 3.31 (SD = 0.84), which falls in the 'Great Extent' range, indicates that community

education programmes improve the status of rural women for improved maternal health in Enugu State to a great extent.

Hypothesis Testing

H₀₁: There is no significant relationship between community education programmes and poverty alleviation for improved maternal health of rural women in Enugu State.

Table 4: PPMC analysis of the relationship between community education programmes and poverty alleviation (n = 385)

Variable Pair	N	r	p-value	df	Decision
Community Education × Poverty Alleviation	385	0.621**	0.000	383	Reject H ₀₁

**** Correlation is significant at the 0.05 level (2-tailed).**

Table 4 shows a Pearson correlation coefficient of $r = 0.621$ and a p-value of 0.000, which is less than the 0.05 level of significance. Consequently, H₀₁ is rejected. This means there

is a significant positive relationship between community education programmes and poverty alleviation for improved maternal health of rural women in Enugu State.

H₀₂: There is no significant relationship between community education programmes and literacy promotion for improved maternal health of rural women in Enugu State.

Table 5: PPMC analysis of the relationship between community education programmes and literacy promotion (n = 385)

Variable Pair	N	r	p-value	df	Decision
Community Education × Literacy Promotion	385	0.714**	0.000	383	Reject H ₀₂

**** Correlation is significant at the 0.05 level (2-tailed).**

Table 5 shows a Pearson correlation coefficient of $r = 0.714$ and a p-value of 0.000, which is less than the 0.05 level of significance. Consequently, H₀₂ is rejected. This means there

is a significant positive relationship between community education programmes and literacy promotion for improved maternal health of rural women in Enugu State.

H₀₃: There is no significant relationship between community education programmes and improvement in the status of rural women for improved maternal health in Enugu State.

Table 6: PPMC analysis of the relationship between community education programmes and improvement in women's status (n = 385)

Variable Pair	N	r	p-value	df	Decision
Community Education × Women's Status	385	0.583**	0.000	383	Reject H ₀₃

*** Correlation is significant at the 0.05 level (2-tailed).*

Table 6 shows a Pearson correlation coefficient of $r = 0.583$ and a p-value of 0.000, which is less than the 0.05 level of significance. Consequently, H_{03} is rejected. This means there

is a significant positive relationship between community education programmes and improvement in the status of rural women for improved maternal health in Enugu State.

Discussion of Results

Finding from Research Question One

The finding of research question one revealed that community education programmes promote poverty alleviation for improved maternal health of rural women in Enugu State to a very great extent (cluster mean = 3.51, SD = 0.74). The PPMC analysis further confirmed a significant positive relationship between community education and poverty alleviation ($r = 0.621$, $p = 0.000$), leading to the rejection of H_{01} . This finding implies that rural women who participate in community education programmes are better positioned to acquire vocational skills, access micro-credit information, and connect with government health financing schemes — all of which directly reduce the financial barriers to maternal healthcare. This finding is consistent with the position of UNDP (2016) that poverty encompasses human deprivation in knowledge and health, and that addressing educational deficits can interrupt the cycle of material deprivation. The finding also aligns with

Chinna, Dokubo, Taylor and Longjohn (2024), who observed that basic literacy and vocational education programmes contributed to reduced maternal and child mortality in Rivers State by enabling women to economically support their health needs. Furthermore, the result corroborates the Health Belief Model (Rosenstock et al., 1950s), which holds that perceived barriers — including the financial cost of healthcare — significantly inhibit health-seeking behaviour. When community education programmes reduce poverty, they simultaneously lower these perceived barriers, thereby encouraging more consistent use of maternal health services. The cluster mean of 3.51 affirms that respondents strongly agreed that economic empowerment activities embedded in community education translate into improved access to antenatal services, skilled delivery care, and postnatal support.

Finding from Research Question Two

The finding of research question two revealed that community education promotes literacy for improved maternal health of rural women in Enugu State to a very great extent (cluster mean = 3.61, SD = 0.68). The PPMC analysis also established a significant positive relationship between community education and literacy promotion ($r = 0.714$, $p = 0.000$), leading to the rejection of H_{02} . This is the strongest correlation observed among the three hypotheses, underscoring the centrality of literacy in improving maternal health outcomes. The finding suggests that community education programmes markedly improve rural women's capacity to read and interpret health information, recognise obstetric danger signs, and appropriately utilise antenatal and postnatal care services. This result is supported by Demie, Gessese, Woldeamanuel and Biratu (2023), who found that literate women demonstrate significantly higher utilisation of maternal healthcare services than their illiterate counterparts. Similarly, Kassim et al. (2023) found that community-based health education facilitated by trained facilitators improved women's knowledge of obstetric danger signs and encouraged greater uptake of maternity care services. Yoseph et al. (2024) further corroborated this finding by demonstrating that community-based health education led by women's groups significantly increased

antenatal care utilisation and health facility delivery rates in Southern Ethiopia. The finding is consistent with the Critical Pedagogy/Empowerment Theory (Freire, 1970), which posits that education creates critical consciousness enabling individuals to identify their problems and take transformative action. When rural women become literate through community education, they gain the awareness needed to make evidence-based decisions about their reproductive health, reducing their susceptibility to misinformation and harmful traditional birth practices.

Finding from Research Question Three

The finding of research question three revealed that community education improves the status of rural women for improved maternal health in Enugu State to a great extent (cluster mean = 3.31, SD = 0.84). The PPMC analysis confirmed a significant positive relationship between community education and improvement in women's status ($r = 0.583$, $p = 0.000$), leading to the rejection of H_{03} . While the mean score fell within the 'Great Extent' rather than 'Very Great Extent' band, the statistically significant correlation underscores the meaningful contribution of community education to women's empowerment. The slightly lower cluster mean relative to those of research questions

one and two suggests that while community education programmes are effective in improving women's status, the deeply entrenched cultural and patriarchal norms in Igbo traditional society present a more complex and persistent challenge. This is consistent with the observation in the problem statement that Igbo traditional societies typically place women in subservient positions, limiting their agency over reproductive and health decisions. The finding aligns with Shimamoto and Gipson (2015), who established that women's status — encompassing their educational, cultural, economic, legal, and political position — is a key determinant of their access to skilled birth attendance. When community education programmes challenge restrictive norms, build women's confidence, and expand their participation in community decision-making, the resulting elevation in women's status creates conditions in which maternal health decisions are made more freely and competently. This finding further corroborates the Critical Pedagogy/Empowerment Theory (Freire, 1970), which emphasises that genuine educational transformation must challenge systemic oppression and empower marginalised groups to act in their own interest. The significant correlation ($r = 0.583$) suggests that, notwithstanding cultural inertia, community education

programmes are making measurable progress in improving the sociocultural and economic status of rural women in Enugu State.

Conclusion

This study investigated the extent to which community education programmes improve the maternal health of rural women in Enugu State, Nigeria, with specific reference to poverty alleviation, literacy promotion, and improvement in women's status. The findings consistently demonstrate that community education programmes exert a significant positive influence on all three dimensions of maternal health improvement. The very great extent to which community education promotes poverty alleviation and literacy, and the great extent to which it elevates women's status, collectively indicate that well-designed and context-responsive community education is a viable and necessary strategy for reducing maternal mortality and morbidity in rural Nigeria.

The significant positive correlations recorded across all three hypotheses confirm that community education programmes are not merely supplementary interventions but are central to addressing the structural determinants of poor maternal health — namely poverty, illiteracy and gender inequality. The findings are

theoretically grounded in both the Health Belief Model, which foregrounds the role of perceived barriers and benefits in health-seeking behaviour, and Freire's Critical Pedagogy/Empowerment Theory, which situates education as a catalyst for social transformation. Together, these theoretical lenses affirm that community education, when delivered in a participatory and culturally sensitive manner, has the potential to transform rural women from passive recipients of healthcare into informed, empowered agents of their own reproductive wellbeing.

The evidence from this study further reinforces the imperative for accelerating progress towards SDG 3 through education-led interventions targeted at rural populations. With Nigeria accounting for a disproportionate share of the global maternal mortality burden, the findings present a compelling case for scaling up community education programmes as a frontline strategy in the national response to maternal health challenges.

Recommendations

Based on the findings of this study, the following recommendations are made:

1. The Enugu State Government and relevant non-governmental organisations should strengthen community education

programmes that incorporate vocational skills training and financial literacy, as these directly contribute to poverty alleviation and improved access to maternal healthcare services.

2. Health education components of community programmes should be prioritised and sustained, given that literacy promotion recorded the strongest relationship with maternal health improvement. Curricula should include specific content on antenatal care, obstetric danger signs, skilled birth attendance, and postnatal care.

3. Community education programmes should adopt participatory and gender-transformative approaches that explicitly challenge harmful cultural norms and build the confidence of rural women to assert their reproductive rights. Engagement of community leaders, traditional rulers and religious leaders is essential to sustainable change in women's status.

4. Adequate funding should be channelled towards adult and non-formal education centres in rural Enugu State to ensure the continuity and quality of community education programmes targeting women of childbearing age.

5. Future studies should explore the longitudinal impact of community education programmes on maternal

mortality rates in Enugu State, using a mixed-methods approach to capture both

quantitative trends and qualitative experiences of rural women.

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